جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سهير عبد المحسن محمد ركش
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** =
رقم البطاقة : 26403110101665 **رقم التليفون :** 01270658831 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 11-03-1964 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سهير عبد المحسن محمد ركش
National ID: 26403110101665

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

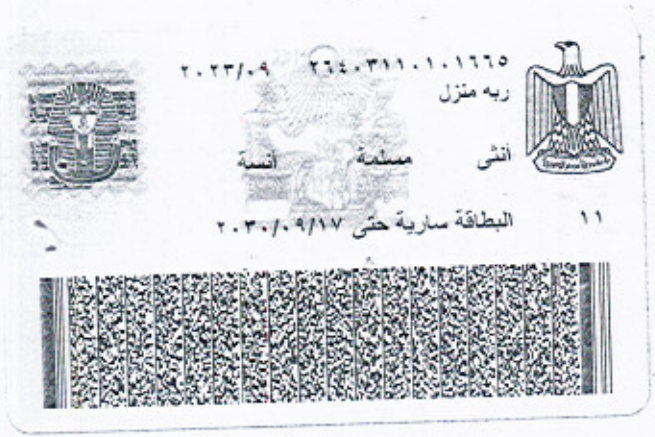
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
سهير
عبدالمحسن محمد بربيش
١٠ش السلام -بيجام
شبرا الخيمة أول - القليوبية
٢٦٤ ٠٣ ١١ ٠١ ٠١٦ ٦٥
KE3153819


٢٦٤٠٣١١٠١٠١٦٦٥ ٢٠٢٣/٠٩
ربه منزل
أنثى مسلمة أنسة
١١ البطاقة سارية حتى ٢٠٣٠/٠٩/١٧

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سهير
عبدالمحسن محمد بربيش

١٠ش السلام -بيجام
شبرا الخيمة أول - القليوبية

٢٦٤٠٣١١٠١٠١٦٦٥

KE3153819



٢٦٤٠٣١١٠١٠١٦٦٥ ٢٠٢٣/٠٩
ربه منزل
انثى مسلمة أعزب

البطاقة صالحة حتى ٢٠٣٠/٠٩/١٧

Name: سهير عبد المحسن محمد بربيش **MR. :** 6641	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented for continuation of letrozole (Femara) and supportive therapy for three months following breast surgery performed on 15-01-2026. The left mastectomy bed and axilla were examined, with a level 2 lymph node noted that displayed nonspecific sonographic features. Follow-up ultrasound was planned in three months.

The patient returned for letrozole and supportive care, reporting severe left arm pain. Physical examination revealed tenderness over bone, prompting a recommendation for a bone scan.

Bone scan was performed; examination demonstrated poor muscle tone, decreased power, and impaired motor function of the left arm, which was numb and painful but retained sensation. The patient was referred for physiotherapy and a neurology consultation.

A CT of the chest, abdomen, and pelvis performed in August 2026 was unremarkable. MRI of the left shoulder showed tenosynovitis and bursitis; the patient was referred to rheumatology and continued physiotherapy. Letrozole was continued for an additional three months with supportive therapy, and zoledronic acid (Zometa) was scheduled for October 2026 with laboratory studies ordered.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **Reham Ahmed Fathy Ibrahim**

يعتمد ,

مستشفى أورام دار السلام
قسم الباثولوجي

Pathology Report

Name: سهير عبدالمحسن محمد بريش Age:56 years.

Clinical data: Invasive duct carcinoma grade 2.

Specimen type: Left modified radical mastectomy (MRM)

Date (Received):28/3/2021 Date(reported): 12/4/2021

Lab. Number:647/21 Hospital Number: 3125/21

Gross Description:

Left modified radical mastectomy (MRM) measuring 21x16.5x4cm with covering skin surface area measuring 18.5x8.5cm including nipple and areola. Cut section revealed a hard greyish white retroareolar mass measuring 4.5x4.5x2.5cm, reaching the nipple and areola grossly, away 0.5cm from the deep margin.
Axillary dissection revealed 21 lymph nodes.

Microscopic picture:

Histopathological examination of sections prepared from the lumpectomy specimen received reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. There are focal insitu ductal components (non comedo +/-10%). positive perineural invasion, positive lymphvascular emboli. Positive malignant deposits in (8/21) axillary lymph nodes with focal extracapsular extension.

Diagnosis:

Left modified radical mastectomy (MRM):

Invasive duct carcinoma grade 2
Partial pathological response (PPR).
Tumour necrosis index +/- 35%
Focal insitu ductal components (non comedo +/-10%).
Positive perineural invasion, positive lymphvascular emboli.
Positive malignant deposits in (8/21) axillary lymph nodes with focal extracapsular extension.
Stage: ypT2, ypN2, ypMx.

Prof. Dr. Lubna Omar
Dr. Abdulrahman Ibrahim

Ministry of health
Central health laboratories
Pathology department

Histopathology report

Report Number: 384

اسم المريض	:	سهير عبد المحسن محمد	
السن	:	52 Y	
تاريخ اخذ العينة	:		النوع : Female
جانب التحليل	:	مركز صحه المراة	العنوان : القاهرة
جهة	:	مركز صحه المراة	تاريخ استلام العينة : 19/07/2020 13:46
			تاريخ التقرير : 20/07/2020 13:58

Primary sample :
left breast core biopsy

comment :

Gross examination :
6 cores of greyish white tissue (T.I)

Microscopic examination :
cores of breast tissue involved by invasive neoplasm of malignant ductal cells arranged in sheets, cords and trabeculae. Malignant cells display moderate cell anaplasia and low mitotic index was observed

Referal Lab :

Comment :

Diagnosis :
LEFT BREAST CORE BIOPSY: THE PICTURE IS COMPATIBLE WITH INVASIVE DUCT CARCINOMA , GRADE II

Lab Director: د/ تريزا ماهر

Pathologist: د/ أنيس رمضان

19 El-sheikh Rehan Street, Abeldeen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website:http://www.healthlabs.net
email:pathologychl@yahoo.com
facebook: قسم الباثولوجى - الادارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بعينة لمدة 30 يوم من تاريخ استلامها

Name : Sohir Abdel Mohsen Mohammed

Date : 13-8-2020

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. **Chest:**

- Left breast heterogeneously enhancing parenchymal mass lesion is seen measuring 5 x 4 cm and inseparable from the nipple-areola complex. Other parenchymal smaller enhancing lesions are also seen.
- Multiple pathologically enlarged left axillary (level I and II) lymph nodes are seen , largest measures 2x 1.2 cm.
- Prominent left internal mammary LN is seen measuring 8 mm.
- Right apical tiny pulmonary nodule is seen measuring 4 mm (for follow up).
- Otherwise, no sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion. Few tiny pleural tags are noted.
- Borderline cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. **Abdomen and pelvis:**

- The liver is of enlarged with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.
- The scanned skeletal portions revealed no gross suspicious lytic or sclerotic osseous lesions.

OPINION:

- ***Left breast heterogeneously enhancing parenchymal mass lesion along with left axillary lymphadenopathy as described.***
- ***Fatty hepatomegaly.No focal lesions.***

Dr. Amr Farouk, MD

Dr. Peter Fahmy, MSc

...ter of Radiation Oncology and
Nuclear Medicine

Faculty Of Medicine, Cairo University

"HEMROCK"

مركز قصر العيني

...لاج الأورام والطب النووي

...لية الطب - جامعة القاهرة

(نفروك)

Report

Patient Name:	soheer Abd El-Mohsen mohammad بسريش	Patient Age:	2019
...rview Code:	Q25356	DATE	10/20/2020 11:25:11 AM

...October 2020

Tc99m MDP SKELETAL SCINTIGRAPHY

Clinical history

- 53-year-old female patient with history of left breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

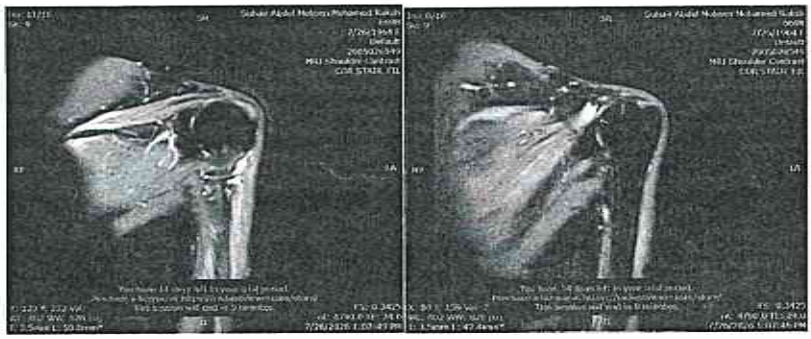
- No abnormal osseous tracer uptake is seen that would account for bone deposits.
- Mild degenerative changes of the spine are noted.
- Osteoarthritic changes of the both knees are seen.

Opinion

- The surveyed skeleton is free from metastatic osseous lesions.

Dr. Ahmed Tawakol, MD

Patient's Name:	**Suhair Abdel Mohsen Mohamed Raksh**
Date:	**2026-07-26**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



Left shoulder MRI without and with contrast.

Technique:

-Multiplanar MRI on T1 and T2WIs and STIR.

-Multiplanar MRI on T1 with contrast.

Findings:

-Mild left gleno-humoral joint effusion.

-No left acromio-clavicular joint effusion.

-A slit-like left subacromion-subdeltoid fluid and a subcoracoid cyst is also seen,and measures about 18x15mm in dimensions.

-Mild high signals are seen in the left supra-spinatus muscle tendon with intact outlines and being surrounded with a slit-like fluid.

-Mild amount of fluid is seen around the intact tendon of long head of left biceps muscle in the bicipital groove.

-Normal MRI of the scanned muscles, bone marrow and bones. No abnormally-enhancing lesions noted.

-No gross subcutaneous anomalies seen.

OPINION:

-Mild left gleno-humoral synovial effusion.

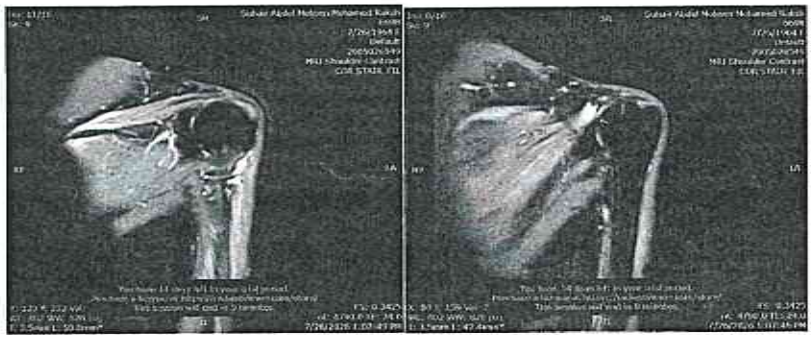
-A slit-like left subacromion-subdeltoid and mild subcoracoid bursitis.

-Mild tendinopathy and teno-synovitis of the left supra-spinatus muscle tendon .

-Mild teno-synovitis of the tendon of long head of left biceps muscle.

Prof/Hanan El-Ahwal.

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Date:	**2026-07-26**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



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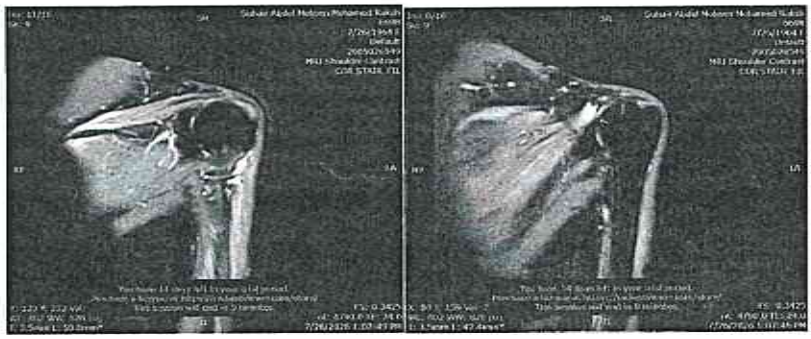
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Patient's Name:	**Suhair Abdel Mohsen Mohamed Raksh**
Date:	**2026-07-26**
Referred by:	Prof.Dr. **no doctor**
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Prof/Hanan El-Ahwal.

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